



GUIDELINE

Mental Health Smoking Care

Scope (Staff): All NMHS Mental Health (MH) Staff

Scope (Area): All NMHS Mental Health Areas

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1. Aim

This document provides guidance for staff caring for patients of the NMHS Mental Health (MH) Service from their initial assessment to discharge and beyond, in order to effectively address nicotine dependence in a supportive and evidence based manner. All staff have a responsibility for their own health and the health of others on the ward by supporting compliance with this guideline.

2. Background

Cigarette smoking is the leading preventable cause of death in Australia. People with mental illness have rates of smoking 2-3 times higher than that of the general population and are more likely to experience ill health and death as a result of smoking related illnesses. While people with mental illness who smoke have higher levels of nicotine dependence, they are just as likely to want to address their smoking behaviours and just as likely to alter their tobacco use with appropriate support as the general population.

People with mental illness are less likely to be offered assistance with their nicotine dependence, making an inpatient admission the opportune time to give people a chance to address their smoking status.

Involuntary inpatients at Mental Health Units (MHUs) in Western Australia are no longer exempt from the [Smoke Free WA Health Policy \(MP 0158/21\)](#). Cigarette smoking and nicotine withdrawal can impact on symptoms of mental illness. ([Refer to DoH Guidelines for the implementation of the Smoke Free Policy](#)).

Behavioural and pharmacological interventions are effective in helping patients admitted to MHUs abstain from smoking and to maintain changes in their smoking behaviour in the long term (Refer to MH Medication: Nurse Initiated Nicotine Replacement Therapy Policy). Not engaging in smoking cessation activities acts as a lost opportunity for significantly improving mental and physical health outcomes.

3. Risk

Risk that the patient will not have nicotine dependence adequately managed whilst an inpatient in a smoke free environment.

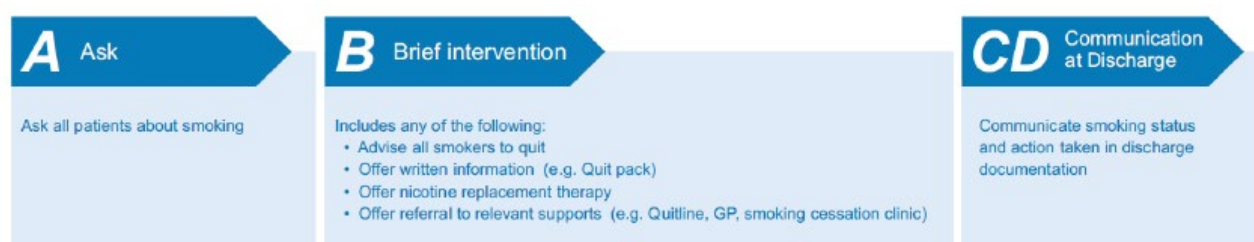
Non-compliance with this guideline will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☐	Policy & Legislative Compliance	☒
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☒	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☒
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☐

4. Model of Practice

This guideline is based on the ABCD¹ approach to supporting people who smoke (Appendix 1). It is a systematic approach designed to be delivered to all people who smoke regardless of intention to quit. It involves identifying the smoking status of every patient and supporting nicotine dependence through to smoking cessation by:

- Asking
- Providing advice (Brief intervention)
- Recording the action taken at the point of hospital discharge (Communication at Discharge).



Source: [An approach to supporting people who smoke: A guide for health services](#)

5. Assessment on Admission

The **NMHS MH INTEGRATED PROGRESS NOTES NRT ASSESSMENT (PMR6)** (Appendix 2) will be completed upon admission. All patients admitted to the ward will be asked about their smoking status, including time to first cigarette, and responses recorded. All staff admitting patients who smoke cigarettes will enquire about past attempts to change their smoking status to determine what was effective or where changes could be made.

Time to first cigarette (TTFC) and number of cigarettes per day (CPD) are measures of nicotine dependence and a useful guide for how much nicotine replacement a patient might need.

All patients will be informed that NMHS MH services are Smoke Free and that a smoking care guideline is in place to ensure nicotine dependence will be managed during their admission. Reassurance and early intervention, e.g. combination Nicotine Replacement Therapy (NRT), will assist both voluntary and involuntary patients to manage this transition. Patients who smoke should also be informed of the Smokers Cessation Clinic at Graylands and referrals for interested patients should be made as early as possible to discuss strategies and treatment beyond behavioural strategies and NRT (e.g. varenicline and bupropion).

Patients who use e-cigarettes (vaporisers) also require assessment of their nicotine dependence. E-cigarettes are not permitted on NMHS MH services.

As with other patient property, packets of cigarettes or tobacco, matches, lighters and vaporisers will be noted on admission and stored for patients until discharge. Patients returning from leave will be encouraged to leave these items at home or to give them to friends or family. Lighters / matches are not to be retained by patients and will not be kept on the ward. Cigarettes and lighters / matches may be stored in patient lockers where these are provided and accessed by patients when leaving the site.

6. Nicotine Withdrawal

Recognising the signs of nicotine withdrawal is important as they can be misinterpreted as symptoms of mental illness. Promptly recognising and managing nicotine withdrawal improves a patient's wellbeing and enhances their ability to engage in other therapeutic activities.

Nicotine withdrawal may be recognised by two or more of the following within 1-24 hours of a nicotine reduction or smoking cessation, and can last up to 2-4 weeks:

- Anxiety
- Irritability or restlessness
- Reduced concentration
- Tobacco craving
- Malaise
- Increased cough
- Dysphoria
- Mouth ulceration
- Insomnia
- Increased appetite

7. Management of Nicotine Dependent Inpatients

To be read in conjunction with [Smoke Free WA Health Policy \(MP 0158/21\)](#) and [the DoH Guidelines for the implementation of the Smoke Free Policy](#).

8. Nicotine Replacement Therapy NRT

NRT aims to replace the nicotine from smoking cigarettes with safer alternatives in order to reduce the symptoms of nicotine withdrawal. Use of NRT over months can down regulate nicotine receptors in the brain, making it easier for patients to then cease the use of NRT over time.

Clinical staff will offer NRT for all patients with nicotine dependence. Patients with moderate to high nicotine dependency are likely to benefit most from combination NRT. This is the provision of fast acting products (such as mouth spray, lozenge or inhalator) combined with nicotine patches. It can enhance chances of quitting and is appropriate if the patient continues to experience withdrawal symptoms or has difficulty abstaining from smoking while on the nicotine patch alone. (Refer to **Appendix 3** for NRT product information).

9. Key Points

- Elimination half-life of nicotine is 40-120 minutes, meaning patients will seek to smoke unless their need for nicotine can be provided in this time. Short acting nicotine may need to be provided during the admission process.

- Nurses and doctors share the responsibility for making sure a patient's NRT needs are adequately covered. Registered Nurses may initiate NRT via the Nurse Initiated NRT Policy (see **Appendix 4**), while doctors are responsible for charting adequate NRT on review, throughout the admission and before discharge.
- The nurse providing NRT will advise patients of the correct use of different NRT products and reminders of this will be placed on the ward. Written information and instructions will be provided to all patients using NRT (**Appendix 3**).
- All patients who choose to accept NRT will be prescribed NRT if they smoke cigarettes, regardless of their intent to smoke when discharged from the ward. Nicotine patches will be prescribed on the regular medication chart (PMR60A) and fast acting products (mouth spray, lozenges and inhalators) will be prescribed on the Nicotine Replacement Therapy (NRT) PRN Chart (PMR56).
- Patients with high level nicotine dependency may find that a single patch and use of one form of short acting nicotine will be inadequate to cover their replacement needs. If patients are reporting ongoing cravings or withdrawal, or these are observed by staff, then additional nicotine replacement will be offered. (See Flow Chart **Appendix 5**)
- Monitoring cravings for smoking cigarettes provides an assessment of the effectiveness of combination NRT. Nursing staff and Doctors will titrate NRT dosing upwards if patients continue to experience cravings.
- Patients going on leave will be offered NRT to take with them to support the management of nicotine withdrawal while on leave. Patients on overnight leave will be prescribed NRT along with other leave medication. Where patients take leave from the ward for short periods, whether leaving the grounds or not, a Registered Nurse can supply immediate release NRT from ward Imprest stock.
- Some patients may be concerned about becoming dependent on forms of NRT. Advice will be provided that the nicotine in NRT is to replace that from cigarettes. This is a harm minimisation strategy that is safe to continue for as long as needed.

Note: For patients who indicate they are not ready to give up smoking, clinical staff will still offer NRT. Evidence suggests that using a nicotine patch whilst smoking either as a 'pre quit' tool or to 'cut down to stop' can improve rates of cessation. Carbon monoxide monitors that measure expired carbon monoxide levels are useful to demonstrate the reduction in smoking to the patient and to verify that progress.

10. Other Pharmacotherapies

Pharmacotherapies for smoking cessation that are not NRT may only be prescribed to patients by the treating doctor. The two most effective forms of non-nicotine pharmacotherapies, Bupropion and Varenicline are summarised below.

10.1 Bupropion

Bupropion (Zyban®) is an oral non-nicotine therapy which affects neuronal re-uptake of noradrenalin and dopamine. Bupropion reduces withdrawal symptoms and has found to be of similar efficacy to NRT. In the community, bupropion is available as a Pharmaceutical Benefits Scheme (PBS) Authority item for the treatment of nicotine dependence in those who are



committed to quitting smoking, when used in conjunction with counselling for smoking cessation / abstinence. In the inpatient setting bupropion is listed on Formulary One.

Discuss with the pharmacist prior to initiating treatment as this may not be suitable for some patients.

10.2 Varenicline

Varenicline (Champix®) was developed specifically to help people stop smoking. Varenicline works by binding to nicotine receptors in the reward centres in the brain. In doing so, it reduces the severity of tobacco withdrawal symptoms while simultaneously reducing the rewarding effects of nicotine. Varenicline is PBS listed as an Authority item for people in a comprehensive support and counselling program for smoking cessation. Varenicline at standard dose increased the chances of successful long-term smoking cessation between two- and threefold compared with pharmacologically unassisted quit attempts. There are reports of psychiatric adverse events with the use of varenicline.

Discuss with the pharmacist prior to initiating treatment as this may not be suitable for some patients.

10.3 Drug Interactions in Smoking Cessation

Products in tobacco smoke affect CYP450 enzymes (specifically 1A2) and affect drug clearance. Patients reducing or ceasing their tobacco use may need medication adjustments, generally dose reduction. NRT, varenicline and bupropion do not affect hepatic clearance in this way but may have other drug interactions. Common medication interactions with smoking cessation are listed in Appendix 6.

Clozapine is particularly sensitive to changes in levels of tobacco smoking and this interaction will be discussed with patients both when commencing clozapine and before discharge. Clozapine blood levels will be assessed 1 week post smoking cessation and patients will be monitored for adverse effects of clozapine such as hypersalivation, tachycardia and oversedation. Smoking status is part of standard clozapine initial workup and review.

Caffeine levels may increase significantly post smoking cessation and effects may be mistaken for nicotine withdrawal. Patients will be advised to reduce caffeine intake and will be monitored for caffeine effects such as irritability, headache or tachycardia.

11. Behavioural Support Interventions

Behavioural support interventions are non-pharmacological interventions designed to assist people in making changes to their level of tobacco use. They may take the form of advice, encouragement, discussion or distraction activities. Evidence based non-pharmacological interventions that can be offered on the ward include:

- Encouragement and support to makes these changes possible
- Education related to the nature of cravings and 'urge surfing' cravings.
- Using strong mints or other sweets. Note that these need to contain sugar to combat cravings so may not be suitable for patients with diabetes.
- Engaging in brief periods of exercise.

- Consider referral to OT for sensory modulation
- Using progressive muscle relaxation or other relaxation techniques.
- Discussing the benefits to that individual of addressing their tobacco use.
- Acknowledging that it can be difficult to make changes around tobacco use.
- Providing encouragement to continue and validating changes already made.
- Breakdown the financial cost of smoking and what else the money could be spent on.
- Identification of cues that trigger cravings for an individual via coping and safety tool
- Referral to patient support group.

12. Approaching People Non-adherent with the Smoking Care Plan

Given the previous smoking exemption for MHUs in regards to the [Smoke Free WA Health Policy \(MP 0158/21\)](#) there may be times when patients, visitors, contractors or staff smoke in areas where this is no longer allowed.

All staff will comply with the [Smoke Free WA Health Policy \(MP 0158/21\)](#) and approach patients and others to address smoking. The key steps to addressing this are:

- Assume the individual is unaware of the policy.
- Politely inform them that the ward/grounds are smoke-free and to refrain from smoking whilst on the grounds.
- Patients and staff will be advised of assistance available to them if they are craving a cigarette or are in nicotine withdrawal and then offer either NRT, behavioural intervention or both.
- As in any situation staff safety is paramount and if there is any concern about aggression the individual will be approached by two staff members and any aggression will be managed with verbal de-escalation before further action is taken in line with the *WATAG Guidelines for Pharmacological Management of Acute Behavioural Disturbance and local practice (under development)*.

13. Smoking During Ward Leave

Patients may have leave from the ward for appointments, time with family, for recovery related activities or community re-integration. During this time off hospital grounds the choice to smoke is an individual one. In order to offer patients the best chance to continue to address their changes in smoking behaviour staff will:

- Offer encouragement to continue abstinence off the ward;
- Offer short acting NRT to take on leave;
- Advise that nicotine withdrawal can be exacerbated if smoking intermittently.
 - Leave from the ward specifically to smoke is against the spirit of the Smoke Free WA Health Policy as it negates the physical and mental health benefits of going smoke



free, increases the symptoms of nicotine withdrawal with intermittent smoking and increases the risk of clinical incidences off the ward during unsupervised leave. Specific 'smokers leave' has no place in a smoke-free health service.

- Smoking will not be permitted on organised outings from the ward and patients will be encouraged to take a short acting NRT with them.

14. Staff Facilitating Smoking

Staff are not permitted to assist patients to leave NMHS MH premises to smoke. Patients are not permitted to smoke in the company of NMHS MH staff.

15. Discharge Planning

With a smoking care approach, an admission to NMHS MH is an opportunity for patients to address their smoking behaviour. To maintain the benefits there will be planning for continuation of care for the patient upon discharge. This includes:

- Discussion with the patient regarding the benefits of continuing to reduce or cease tobacco use.
- Recognition of cues to smoke and ways to manage cues and cravings.
- Offering short and long acting NRT and/or pharmacotherapy to continue at home as part of the discharge prescription.
- Including the smoking cessation actions taken, including amount of NRT required, in the discharge summary.
- Referral to additional resources, such as Quitline 13 78 48.
- Adding smoking related diagnoses to the discharge summary including advice to General Practitioners for continuation of care and options.

16. Nicotine Dependent Staff

A portion of staff working on a smoke-free ward will themselves be dependent on nicotine. Smoking cessation interventions including NRT are available free of charge to NMHS staff and staff are encouraged to access this service.

For staff who continue to smoke the following will be considered:

- Staff need to leave hospital grounds to smoke;
- Regularly scheduled breaks may be used for smoking, but as per the *WA Health Smoke Free Policy* 'smoke breaks' are not permitted in addition to these;
- Staff will be mindful of the effect that residual toxins and the smell of cigarettes may have on a smoke free ward;
- Staff ignoring the [Smoke Free WA Health Policy \(MP 0158/21\)](#) may be reported for misconduct.

17. Compliance, monitoring and evaluation

- Annual Documentation Audit of compliance with policy (SQPU)
- Annual Report provided by Public Health
- NMHS MH Annual Pharmacy report on NRT
- Mandatory e-Learning on Smoke Free Environment in MHPHDS Corporate Orientation

Related internal policies, procedures and guidelines

[Smoke Free WA Health Policy \(MP 0158/21\)](#)

[DoH Guidelines for the implementation of the Smoke Free Policy](#)

[MH Medication: Nurse Initiated Nicotine Replacement Therapy Policy](#)

References

[National Tobacco Strategy 2022-2030](#)

[WA Health Promotion Strategic Framework 2022-2026](#)

[National guidance note on the elimination of environmental tobacco smoke in the workplace \[NOHSC:3019 \(2003\)\]](#)

[Christensen, D. 6.11 Tolerance, Dependence, and Withdrawal. In Scollo, MM and Winstanley, MH \[editors\]. Tobacco in Australia: Facts and issues. Melbourne: Cancer Council Victoria; 2018. Available](#)

[NMHS Smoke Free Hub](#)

[Quitline Health Professionals](#)

[NMPHDS Mental Health Smoke Free Staff Survey Fact Sheet](#)
<https://mhphds-healthpoint.hdwa.health.wa>

Useful resources

[Mental Health Framework for State-wide Standardised Clinical Documentation \(SSCD\) Triage to Discharge vs.11](#)

[PMR 5 AUDIT and DAST\)](#)

[The Alcohol, Smoking and Substance Involvement Screening Tool \(ASSIST\) SMHMR966\)](#)

[Nicotine Replacement Therapy \(NRT\) PRN Chart \(PMR56\).](#)

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Policy Contact	Director of Nursing MHPHDS				
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Approved by:	Director of Nursing MHPHDS			Date:	13/07/2022
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NSQHS Standards Applicable:	☒ Std 1: Clinical Governance ☐ Std 2: Partnering with Consumers ☐ Std 3: Preventing and Controlling Healthcare Associated Infection ☐ Std 4: Medication Safety ☐ Std 5: Comprehensive Care ☐ Std 6: Communicating for Safety ☐ Std 7: Blood Management ☐ Std 8: Recognising and Responding to Acute Deterioration				
National Standards for Mental Health Services	☒ Std 1: Rights and Responsibilities ☐ Std 2: Safety ☐ Std 3: Consumer and Carer Participation ☐ Std 4: Diversity Responsibility ☐ Std 5: Promotion and Prevention ☐ Std 6: Consumers ☐ Std 7: Carers ☐ Std 8: Governance, leadership and management ☐ Std 9: Integration ☐ Std 10: Delivery of Care ☐ 10.1 Supporting Recovery ☐ 10.2 Access ☐ 10.3 Entry ☐ 10.4 Assessment and Review ☐ 10.5 Treatment and Support ☐ 10.6 Exit and Re-entry				
Chief Psychiatrist's Standards for Clinical Care:	Physical Health Care of Mental Health Consumers,				
Printed or personally saved electronic copies of this document are considered uncontrolled					

Document Version Control			
Date	Version	Author	Notes
08/07/2019	1.0	Chief Pharmacist NMHS MH	Initial endorsement
25/10/2019	1.1	Chief Pharmacist NMHS MH	Amended hyperlink to MH Protection of Staff from Environmental Tobacco Smoke in the Workplace to MH Smoke Free Policy
14/07/2022	2.0	Director of Nursing MHPHDS	Scheduled review, change of sponsor, removal of reference to exemption for MH Inpatients, updated references, template and hyperlinks

The health impact upon Aboriginal people have been considered, and where relevant incorporated and appropriately addressed in the development of this health initiative.

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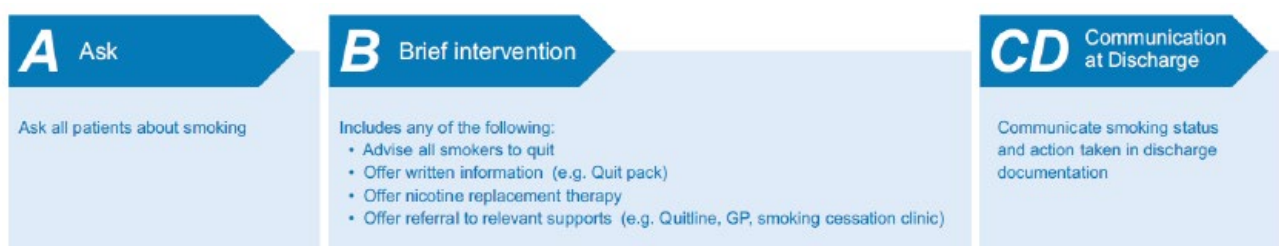
Appendix 1: ABCD Brief Intervention Model

This guideline is based on the ABCD approach to supporting people who smoke. It is a systematic approach designed to be delivered to all people who smoke regardless of intention to quit. It involves identifying the smoking status of every patient and supporting nicotine dependence through to smoking cessation by:

Asking

Providing advice (Brief intervention)

Recording the action taken at the point of hospital discharge (Communication at Discharge).



Assessment : <u>A</u> sk	<u>B</u> rief intervention: <i>Providing advice</i>	<u>C</u> ommunication at <u>D</u> ischarge
Ask all patients (where clinically possible) their smoking status (current smoker, ex-smoker, never smoked) and/or electronic cigarette use during the admitting medication history interview process. Response to be recorded on the Integrated Progress Notes NRT Assessment (PMR6). See Appendix 2	Ideally brief intervention support aims to assist long term quitting. In some cases, brief interventions may be more focused on the temporary management of nicotine withdrawal. Intervention will include the following: - Explain that all indoor and outdoor areas on NMHS MH grounds are totally smoke-free (including the use of electronic cigarettes and personal vaporisers). - Advise all patients who smoke to quit in a clear, non-confrontational way. - Offer nicotine replacement therapy (NRT) See Appendix 3 - Offer referral to relevant supports (e.g. Quitline, general practitioners, community smoking cessation clinics).	Include smoking status, support received and plan for follow up in discharge communication. Ensure prescription of NRT or other related pharmacotherapy on discharge in accordance with current PBS guidelines. All patients are potentially eligible to receive a supply of NRT on discharge. The quantity supplied will match the duration of other discharge medication.



Appendix 2: Nicotine Replacement Therapy Options

NRT Options	Time to Peak Effect	Recommended Dose*	How to Use
Patches	2-6 hours	21mg/ 24 hours	Place a patch on clean, dry, hair free skin. Do not apply straight after a hot shower. Commence patch at night to cover morning cravings for nicotine. Rotate the patch site daily to prevent build-up of adhesive. Nicotine patches should not be used in patients with adverse reactions to adhesive dressings or on areas of eczema or broken skin.
Mist spray	<5 minutes	1-2 sprays Maximum of 4 sprays/ hour	After priming, spray 1-2 pumps of the mist spray towards the inside of cheek or under tongue, avoiding the lips. The nicotine is absorbed through the lining of the mouth. To help absorption try not to swallow for a few seconds. Use with caution in patients with chronic throat conditions or unstable asthma. Patients will be assessed for risk to self and others with regard to keeping the spray on their person.
Lozenge	20 minutes	4mg Maximum one lozenge every 1-2 hours	The lozenge takes about 20 minutes to dissolve. As the lozenge dissolves in the mouth, nicotine is released into the saliva and absorbed through the buccal mucosa. Sucking or chewing on the lozenge does not speed up the nicotine release. The lozenge will not be effective if swallowed. Patients should not eat 15 minutes before use and should not eat or drink with the lozenge in the mouth. Patients over the age of 64, or patients with dementia and other cognitive impairments or on soft diet will be assessed as to their ability to use lozenges appropriately.
Inhalator	20-30 minutes	15mg cartridge Maximum one cartridge every 2 hours	Insert cartridge, close device to puncture. Each cartridge will deliver approximately 80 puffs before it is depleted. Inhale air through cartridge for 20 minutes, breathing in and out through the inhalator in short puffs. Patients should self-titrate dose according to withdrawal symptoms. Use with caution in patients with chronic throat conditions or unstable asthma. The nicotine in the cartridge will evaporate over 2 hours once removed from packaging.

*Patients with high dependence on nicotine may require more than the recommended dose of all forms of NRT



Appendix 3: Nicotine Replacement Flowchart

Nicotine Replacement Flowchart

Assess smoking status by asking if the patient smokes tobacco and time to first cigarette (TTFC) and number of cigarettes per day (CPD) [PMR6]

High

- 21mg/ 24-hour patch
- AND
- Short acting nicotine replacement
 - mouth spray
 - inhalator
 - lozenge

Low

- Short acting nicotine replacement
- mouth spray
- inhalator
- lozenge

May require 21mg in 24-hour patch if cravings persist

Non-smoker

No further action

If ongoing cravings or withdrawal symptoms observed or reported

- Add an additional 21mg/24-hour nicotine patch
- Add additional short acting nicotine replacement, e.g. inhalator, lozenge
- Commence nicotine mouth spray if not already in use
- Ensure the client is using NRT correctly

Ensure non-pharmacological interventions are offered

On discharge

- Offer all patients requiring NRT a supply on discharge of the short and long acting nicotine being used in hospital
- Document nicotine dependence, NRT form and amount used in discharge summary
- Encourage follow up with GP for smoking cessation support

Provide written advice on smoking cessation and encourage the ongoing use of NRT

Appendix 4: Clinically Significant Drug Interactions Associated with Smoking and Smoking Cessation

Drug	Mechanism of interactions and effects with smoking	Clinical Relevance	Nicotine or polycarb	Management in smoking cessation
Caffeine	Caffeine is >99% metabolized through CYP1A2. Increased metabolism; increased clearance (56%).	High	Polycarb	Caffeine levels are likely to increase upon smoking cessation. In most cases caffeine intake needs to be reduced by half to avoid excessive caffeine levels.
Chlorpromazine	Chlorpromazine is principally metabolised through CYP1A2. Decreased AUC (36%) and serum concentrations (24%). Smokers may need higher doses and decreased sedation and hypotension possible in smokers.	Moderate	Polycarb	Be alert for increased adverse effects of chlorpromazine (e.g. EPSE, dizziness, sedation). If adverse effects occur, reduce the dose accordingly.
Clozapine	Clozapine is principally metabolised through CYP1A2. Increased metabolism and decreased plasma concentrations of around 18%. Plasma level reduction may be greater in those receiving valproate.	High	Polycarb	Mean increase of up to 72% in plasma clozapine concentrations. Monitor trough plasma level before stopping smoking and for two weeks after stopping smoking or sooner if side effects develop. Be alert for increased adverse effects and if they occur, reduce the dose accordingly in consultation with psychiatry.
Erlotinib	Plasma levels of erlotinib are decreased in current smokers compared with non-smokers (2-fold). Clearance is increased by 24%.	High		Smokers will be encouraged to stop before erlotinib therapy is initiated. When given to patients who smoke, increase the daily dose in 50mg increments at 2-week intervals, up to a maximum dose of 300mg. If the patient stops smoking the dose will be immediately reduced to the initial starting dose.
Insulin	Possible decrease in insulin absorption secondary to peripheral vasoconstriction. Smoking may also cause release of endogenous substances that cause insulin resistance.	Moderate	Unknown	If a patient with insulin dependent diabetes stops smoking, their dose of insulin may need to be reduced. Advise the patient to be alert for signs of hypoglycaemia and to test their BSLs more frequently.

Methadone	Methadone is metabolised via isoenzymes including CYP1A2 (a minor pathway). There has been a case report of respiratory insufficiency and altered mental status when a patient taking methadone for analgesia stopped smoking.	Moderate	Unknown	Be alert for signs of opioid toxicity and reduce the methadone dose accordingly.
Olanzapine	Olanzapine is principally metabolised through CYP1A2. Increased metabolism; increased clearance (98%) and decreased serum concentrations (12%). Smokers may require higher doses.	Moderate	Polycarb	Be alert for increased adverse effects of olanzapine (e.g. hypotension, sedation and dizziness) and if they occur reduce the dose accordingly. Be alert for increased adverse effects of olanzapine and if they occur reduce the dose accordingly in consultation with psychiatry.
Theophylline	Theophylline is principally metabolised through CYP1A2. Increased metabolism; increase clearance (58-100%) and decrease half life (63%). Heavy smokers (20-40 cigarettes per day) may need much higher doses than non-smokers due to increased elimination rate and shortened theophylline half-life.	High	Polycarb	Monitor theophylline plasma concentrations and adjust the dose accordingly (the dose may need to be reduced up to one third a week after smoking cessation). It is important to continue to monitor theophylline concentration periodically as enzyme induction may take several weeks to normalise. Advise patients to seek medical advice if they develop signs of theophylline toxicity such as palpitations or nausea.
Warfarin	Warfarin's less active R isomer is eliminated to a minor extent by CYP1A2. A recent meta-analysis showed that smoking appeared to increase the warfarin dose requirement by 12% resulting in an extra 2.26mg/week.	Moderate	Polycarb	The INR may increase so the patient needs to be monitored closely. Advise patients to let the doctor managing their anticoagulant control know they are stopping smoking.

* Only the clinically significant interactions are provided here. To search for other drug interactions that have been investigated but are not considered clinically significant please refer the FAQ on DrugNet or contact NMHS MH Drug Information

Appendix 5: NMHS MH Smoking Care Guideline for Patients Admitted via the Emergency Department

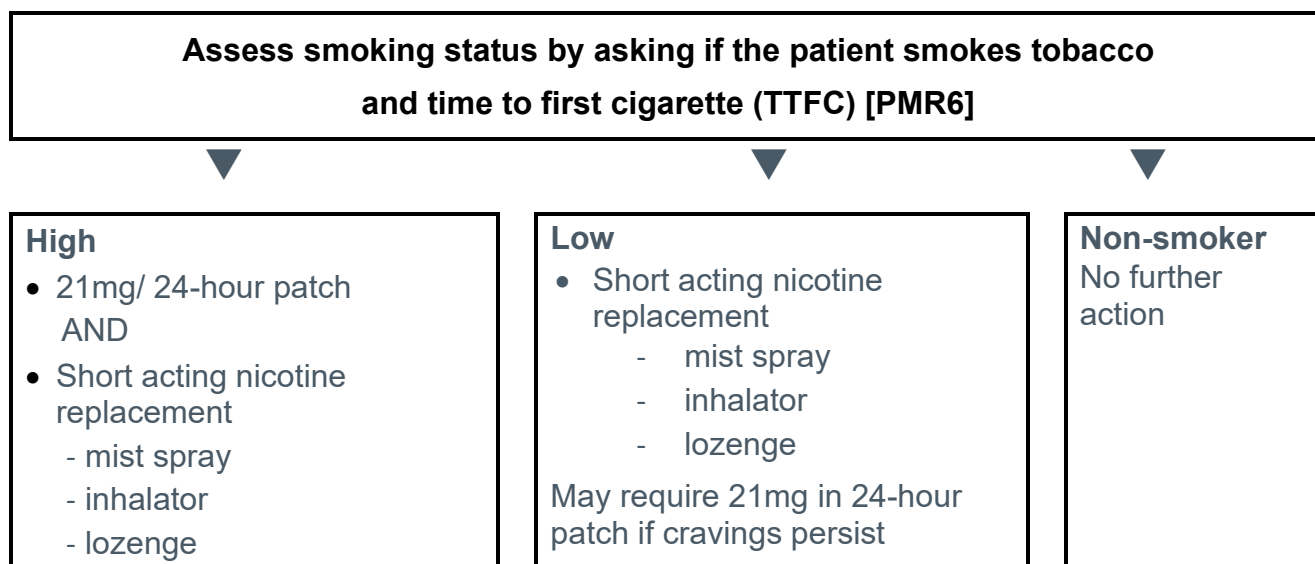
From August 2019 NMHS MH Services are going to be smoke free. This includes MHOA and precludes admitted patients from leaving the ward specifically to smoke cigarettes.

Nicotine withdrawal can commence within an hour of a regular smoker abstaining from tobacco and symptoms can often be confused with symptoms of mental illness, such as

- Anxiety
- Irritability or restlessness
- Reduced concentration
- Tobacco craving
- Malaise
- Dysphoria
- Insomnia
- Increased appetite

Assessment of smoking status and provision of adequate nicotine replacement therapy (NRT) is an important part of care, facilitates assessment and allows for a smoother admission process to the NMHS MH Services.

When a patient presents to the Emergency Department the following flowchart will be used:



If a Mental Health Liaison Nurse is not on duty, please inform the patient of the smoke free policy and ensure nicotine replacement is charted and provided before transfer to NMHS MH Services.